Explore 16.2

Counting in Qualitative Research

Google 'reliable and unreliable numbers'. Now read the following paper to understand how using numbers can work in qualitative research: http://journals.sagepub.com/doi/pdf/10.1177/1056492610375988

This paper reviews why counting is a controversial issue in qualitative research in business studies and explains how this controversy creates a 'multiple audience problem' for qualitative researchers. It goes on to identify the purposes that can be served by four different types of counting.

What is the multiple audience problem?

What are the different types of counting?

In his observation of classroom behaviour, Mehan suggests that many kinds of quantification have only limited value:

> the quantitative approach to classroom observation is useful for certain purposes, namely, for providing the frequency of teacher talk by comparison with student talk ... However, this approach minimizes the contribution of students, neglects the inter-relationship of verbal to non-verbal behavior, obscures the contingent nature of interaction, and ignores the (often multiple) functions of language. (1979: 14)

I do not attempt here to defend quantitative or positivistic research per se. I am not concerned with research designs which centre on quantitative methods and/or are indifferent to how participants construct order. Instead, I want to try to demonstrate some uses of quantification in research which is qualitative and interpretive in design. As the following case study shows, results from quantitative surveys can reveal a phenomenon which qualitative research can explain.

A very nice example of how simple tabulations can improve the quality of data analysis is provided by Ross Koppel et al. (2003). Their earlier ethnographic research had revealed that hospital computer-based ordering systems were often associated with errors when doctors prescribed patients' medications. A quantitative survey now showed that over 75 per cent of doctors had used the computer system incorrectly.

It turned out that the computer display tended to convey a false sense of accuracy to many doctors. For example, by focusing solely on the electronic medication chart, doctors would tend to miss crucial paper stickers attached to the hardcopy casenotes. Various features of the computer software also seemed to be associated with these errors. For instance, the display on the screen would show amounts of a medication appropriate for warehousing needs and purchasing deci- [illegible] clinically inappropriate. In addition, it was possible